

Our Ref: (T) AK/NNP/n/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division A

Care Group - Surgery ENT

A Level, Mailpoint OEN

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME]

Mr [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnosis:

Asymmetrical tonsils plus recurrent sore throats

Management:

Follow up to discuss tonsillectomy

I reviewed this 38-year-old gentleman who works as a project delivery manager for the NHS. He has a long history of recurrent sore throat on a weekly basis. He denies any pharyngeal symptoms. He does not smoke and has no alcohol intake.

He does drink 10 pints of alcohol a week. He has a past history of Crohn's for which he takes azathioprine and omeprazole. He has no allergies.

Examination of the oral cavity shows asymmetrical tonsils with the right greater than left but appearances are normal. Examination of the neck shows a small volume lymph node in the posterior triangle which appears to be reactive.

Considering the fact that he has had this problem on numerous occasions and has seen us previously, he is clearly concerned and the fact that he has got asymmetrical tonsils, I think it will be prudent for him to undergo a bilateral tonsillectomy to make sure there is nothing sinister going on. I have explained to him the additional risks of having a tonsillectomy.

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He would like to think about it and thus I have offered him an appointment in three weeks time to give us his final decision.

Yours sincerely

Electronic signature will go here DO NOT REMOVE

Afroze Khan BSc, MBBS (Lond), MRCS (Eng), DOHNS (Ed)

Specialist Registrar to Mr [NAME]